

ONTARIO INSURANCE COMMISSION

BETWEEN:

RICHARD MARK PLOWS

Applicant

and

JEVCO INSURANCE COMPANY

Insurer

DECISION

Issue:

The Applicant, Richard Mark Plows, was catastrophically injured as a result of a motorcycle accident on October 27, 1990. He fractured his spinal cord at the level of T-8, resulting in complete and permanent sensory-motor paraplegia from his waist downwards. The Applicant is a university student, currently enrolled as an undergraduate at the Faculty of Engineering Science of the University of Western Ontario. He applied for and received personal injury benefits payable under Regulation 273/90 enacted under the Ontario Insurance Act, R.S.O. 1990, chap. 1-8, (the "No-Fault Benefits Schedule").

The Applicant was hospitalized for medical, surgical and rehabilitative care following his accident. His medical and rehabilitative treatment concluded at the end of January, 1991 when he was ready to be discharged from hospital.

The Applicant was not able to be discharged because he lacked suitable accommodation. His parents' home, where he had previously resided, was not

wheelchair accessible. The Applicant and the Insurer failed to agree about the nature and extent of the required modifications to the home.

The Applicant subsequently requested the amount of \$900.00 monthly to rent a wheelchair-accessible apartment unit, pending home renovations. The Applicant also claimed that he required the use of a modified motor vehicle as part of his rehabilitation. The Insurer denied that it was obliged to provide a motor vehicle.

The Applicant applied for mediation on the issue of the home renovations. In a separate application, the Applicant applied for mediation with respect to the issue of the vehicle, and for his rental costs for the apartment.

The parties failed to resolve any of the issues brought to mediation. The Applicant subsequently applied for the appointment of an arbitrator under Section 282 of the Insurance Act, and the hearing of all the issues between the parties was consolidated.

The parties resolved the issue of the apartment rental costs and home renovations prior to the hearing.

The issues to be determined at the arbitration hearing are:

- (1) Is the Applicant entitled to be provided with a suitably modified motor vehicle, as part of his reasonable expenses for rehabilitation?
- (2) Is the Applicant entitled to a special award under Section 282(10) of the Insurance Act, on the basis that the Insurer has unreasonably delayed or withheld payments?

Result:

The decision is:

The Applicant is entitled to be provided with a suitably modified motor vehicle.

The Applicant is not entitled to a special award under Section 282(10) of the Insurance Act.

Hearing:

A hearing was held at London, Ontario on September 23, 1991 and September 24, 1991, before me, Frederika M. Rotter, Senior Arbitrator.

Present at the hearing were:

Applicant: Richard Mark Plows

Applicant's
Representative: Brian A. Foster
Barrister & Solicitor

Insurer's
Representative: John S. McNeil
Barrister & Solicitor

Documents before the Arbitrator:

Exhibit 1 Ontario Automobile Insurance Application for Accident Benefits, dated December 1, 1990

Exhibit 2 Correspondence to Brian Foster, from Dr. Richard Allatt, dated March 5, 1991

Exhibit 3 Correspondence to the Ontario Insurance Commission, from Dr. Richard Allatt, dated April 11, 1991 (current accommodations)

Exhibit 4 Correspondence to the Ontario Insurance Commission, from Dr. Richard Allatt, dated April 11, 1991 (renovations)

Exhibit 5 Correspondence to the Ontario Insurance Commission, from Dr. Richard Allatt, dated April 11, 1991 (motor vehicle)

Exhibit 6 Multidisciplinary Team Report, from Lynda Vokey, dated December 10, 1990

Exhibit 7 Future Care Costs Assoc. Report dated March 1, 1991

Exhibit 8 Future Care Costs Assoc. Report dated May 1991

Exhibit 9 Correspondence to Christine Terashita of Morden & Helwig, from Lynda Vokey, dated December 20, 1990

Exhibit 10 Report of Dr. William O. Geisler, dated May 8, 1991

Exhibit 11	Report of Dr. John C. Clifford, dated July 15, 1991
Exhibit 12	Associative Rehabilitation Inc. Report, dated September 9, 1991
Exhibit 13	Winmar Plan, dated January 31, 1991 (covering letter dated February 21, 1991)
Exhibit 14	Report to Insurer, from Christine Terashita of Morden & Helwig, dated December 4, 1990
Exhibit 15	Memorandum to Insurer, from Christine Terashita of Morden & Helwig, dated December 17, 1990
Exhibit 16	Report to Insurer, from Christine Terashita of Morden & Helwig, dated December 27, 1990
Exhibit 17	Memorandum to Insurer, from Christine Terashita of Morden & Helwig, dated February 5, 1991
Exhibit 18	Memorandum to Insurer, from Christine Terashita of Morden & Helwig, dated February 8, 1991
Exhibit 19	Report to Insurer, from Christine Terashita of Morden & Helwig, dated February 18, 1991, including copy of estimate of First General Services
Exhibit 20	Correspondence to Christine Terashita of Morden & Helwig, from Brian A. Foster, dated March 4, 1991
Exhibit 21	Report to Insurer, from Christine Terashita of Morden & Helwig, dated March 7, 1991
Exhibit 22	Report to Insurer, from First General Services (London), Inc., dated May 3, 1991

The following documents were before the arbitrator, but not marked as exhibits:

- Report of Mediator, dated May 13, 1991
- Second Report of Mediator, dated August 19, 1991
- Application for Appointment of an Arbitrator, dated June 5, 1991, and received on June 18, 1991
- Second Application for Appointment of an Arbitrator, dated August 23, 1991, and received August 26, 1991
- Response of Insurer, filed June 18, 1991

Cases referred to:

Stewart v. Allstate Insurance Company 510 A.2d 1131 (N.J. 1986)

Reed v. Citizens Insurance Company of America (unreported decision of Hon. R.L. Tahoven, issued June 11, 1990, County of Clinton, Michigan)

Manley v. Detroit Automobile Inter-Insurance Exchange 388 N.W.2d 216 (Mich.1986)

The Evidence:

The facts of this case will be summarized briefly before dealing with the evidence of the witnesses. They are as follows:

The Applicant, Richard Mark Plows, is a student of engineering at the University of Western Ontario. Prior to his accident, he lived with his parents in the family home in London, Ontario. The Applicant was injured in an accident on October 27, 1990, when his motorcycle collided with an automobile. The accident left him paralysed from the waist down, and confined to a wheelchair.

The Applicant's father, Robert Plows, contacted the Insurer on November 5, 1990, while the Applicant was still in hospital, to initiate the claim for no-fault benefits. The issue of renovations to the Applicant's family home was first raised in December 1990, when it was contemplated that the Applicant would be temporarily discharged from hospital to spend time with his family during the Christmas holidays. On December 10, 1990, a home assessment was done by a multidisciplinary team from the Parkwood Hospital, including Lynda Vokey, an occupational therapist. It was apparent that the home required modifications in order to make it wheelchair accessible.

At that time, it was not clear when the Applicant would be ready to be discharged from hospital on a permanent basis. Therefore, the modifications that were considered were of a temporary or interim nature.

On December 17, 1990, after she was advised by the Applicant's father that the Applicant was expected to be home for a Christmas visit, the agent of the Insurer,

Christine Terashita, contacted Winmar Industries Inc. ("Winmar"), a general contractor in the London area, with a view to renovating the home to make it wheelchair accessible.

After further discussions between the parties, a meeting was held at the Applicant's home on January 8, 1991, to discuss the issue of the home renovations. The meeting was attended by Ms. Terashita, Robert Plows, Lynda Vokey, the occupational therapist from the hospital and John Williams of Winmar. The barriers to accessibility in the home were discussed, and suggestions for modifications were made. On or about the middle of January 1991, the Applicant returned to his studies at the university, on a reduced schedule.

Winmar produced a report and estimate (the "Winmar plan") for the home renovations on February 5, 1991. This report was faxed by the agent to the head office of the Insurer in Montreal on the same day, and the agent requested authorization for the work to be commenced, on an urgent basis. The Insurer declined to grant authorization in the absence of another competitive estimate. Therefore another contractor was contacted and a second estimate for the work was obtained on February 18, 1991. The Insurer then authorized that work be commenced pursuant to the Winmar plan on February 20, 1991.

While the Insurer was pursuing the home modifications based on the meeting of January 8, the Applicant and his family were attempting to deal with his situation on an ongoing basis. The Applicant was ready to be discharged from hospital by the end of January 1991, much earlier than had been originally anticipated. Since he was attending school, and also had ongoing rehabilitation and medical needs, he required a reliable method of transportation. To help the Applicant plan for and adjust to his new conditions, the Applicant's family retained a private company known as Future Care Costs Associates ("Future Care"), to prepare a comprehensive report outlining the Applicant's care, support and rehabilitation needs.

Future Care produced a first report (the "first Future Care report") on March 1, 1991, dealing with the Applicant's immediate needs for accessible accommodation. The report

outlined two possible scenarios for home modifications. The first (the "A option") was based on the Winmar plan, and involved modifying the existing family room to serve as living space for the Applicant. The second scenario (the "B option") involved building an addition to the home to accommodate the Applicant. The B option was the scenario recommended as the more appropriate.

The Applicant and his family refused to allow renovations to proceed pursuant to the Winmar plan, after consulting with Future Care. The Future Care report was received by the Insurer on or about March 7, 1991. Approximately one week later, the Applicant referred the matter to mediation. The matter was unable to be resolved in mediation and the application for an arbitrator was subsequently filed. In the interim, Future Care prepared a second more comprehensive report, dated May 7, 1991, (the "second Future Care report") that addressed, among other things, the Applicant's transportation requirements, and recommended that he be provided with a suitably modified van.

The Insurer took the position that it was not obliged to provide a vehicle for the Applicant, although it was prepared to pay the costs of modifying a vehicle for the Applicant's use if he bought the basic vehicle himself.

On April 26, the Applicant was finally discharged from hospital into an apartment dwelling which he shared with his sister. He was able to function adequately in the apartment with her assistance and care.

In August, the Applicant initiated another mediation, requesting that he be reimbursed for the rent for his apartment, and also requesting that he be provided with a suitably modified van, pursuant to the second Future Care report. This mediation was also unsuccessful and the parties proceeded to arbitrate all the outstanding issues. Prior to the hearing, the parties reached an agreement about the home renovations, based on figures in the second Future Care report, which provided a considerably lower estimate of the costs involved in the "B Option", the recommended plan for building an addition to the home. The Insurer indicated that it had not been provided with a copy of the second Future Care report until just prior to the arbitration hearing. The Insurer also agreed to

pay the Applicant's apartment rental costs after having been provided with rental receipts.

Dr. Richard Douglas Allatt gave sworn testimony on behalf of the Applicant. He testified that he is a physician, licensed to practice in the Province of Ontario, with a specialty in rehabilitative medicine. He explained that this is a sub-specialty of internal medicine and he has undergone training in managing catastrophic injuries.

The physician outlined his credentials, testifying that he was licensed in 1978. He completed a three-year residency in family medicine and subsequently entered a residency program in rehabilitative medicine, from which he graduated in 1984. After this residency, he took up the position of Chief of Rehabilitation Services at the Parkwood Hospital, where he was in charge of spinal cord injuries. He has co-authored a number of articles with Dr. Richard Hayes of the Faculty of Medicine of the University of Western Ontario, and he also participated in a number of recent projects.

At the Parkwood Hospital, the physician normally had approximately 300 people on his case load, with approximately 80 to 100 new cases every year. The physician was offered as an expert witness and his credentials as such were accepted.

The physician testified that he became involved with the Applicant after his injury. The Applicant was transferred to the Parkwood Hospital on November 26, 1990 after his acute treatment, which included surgery to reconstruct and stabilize his spine and other broken bones.

The physician testified that when the Applicant was transferred to the Parkwood Hospital, he was totally and profoundly dependent for everything, and required pain control.

The Applicant's diagnosis was a T-8 fracture dislocation, which refers to a fracture of the thoracic spine about three-quarters of the way down. As a result of this fracture, the Applicant suffered from T-8 paraplegia, that is total paralysis of sensation and movement below the level of his injury. The physician testified that the T-8 joint is just above the navel, and from this area downwards the Applicant suffers loss of all

movement and also impairment of feeling such as hot and cold sensation. The paraplegia affects the Applicant's bowel, bladder, and sexual functions. The Applicant's spine has been stabilized operatively, but the Applicant can expect no change in the level of his paralysis. His condition is permanent. The physician identified Exhibit 1 to the hearing, which is a copy of the Ontario Automobile Insurance Application for Accident Benefits, which includes a medical report, which he signed. The medical report indicates that the objective findings with respect to the Applicant are T-8 fracture and dislocation and complete T-8 Frankel paraplegia. The report states that the Applicant suffers permanent full disability. At Item 7, in response to the question "what, after discussion, is the estimate when the Claimant will be able to return to work or normal activities", the answer is "NEVER!". The physician testified that he signed and completed the medical report on November 30, 1990.

The physician outlined primary domains or tasks of interest in rehabilitation. They include identifying the injury and its effects, identifying the functions that have been impaired, and helping the injured person return to optimal function in light of his or her real deficits. The following functional areas are addressed: hygiene, eating, mobility, transfers, and dressing. Other domains or areas of interest in rehabilitation include examining and coping with the psychological manifestations, the social circumstances, and the vocational circumstances of the individual.

The physician testified that he was head of the rehabilitation management team at the Parkwood Hospital, a team which included other professionals such as psychologists, physiotherapists, occupational and vocational therapists, social workers and religious counsellors. The physician testified that the minimum goal for the Applicant was functional independence: that is to send him home from his rehabilitation training, able to manage his five basic functions. This does not include further integration into the community and the social aspects of rehabilitation.

The physician testified that decisions regarding the Applicant's rehabilitation program ultimately rested with him, but the Applicant and his family were co-participants in the decision-making process. The physician testified that it costs approximately \$370 to

\$390 a day for basic services to keep the Applicant hospitalized. Therefore, the goal is to discharge individuals from hospital as quickly as possible. In this case, the Applicant had visited home at Christmas, and was approved for discharge by the end of January 1991. At that point in his rehabilitation program he had achieved the basic minimum functional goals. Further rehabilitation for this Applicant would involve helping him integrate into the community in circumstances that are optimal.

The physician testified that the Applicant became eligible for day passes very quickly. With the provision of extraordinary support from his family, the Applicant was able to go home overnight. The physician testified that the normal rehabilitation schedule was accelerated for the Applicant, and he was out rapidly for days in a row. The physician testified that the Applicant's family was essential to his rehabilitation, providing him with both physical and social support.

The physician testified that members of the rehabilitation team attended at the family home to evaluate the assets and barriers in the home, and to fine-tune a rehabilitation program.

The Applicant could not be discharged from hospital until April 26, 1991 because, prior to that time, he could not be accommodated in circumstances that were safe and reasonable. The physician testified that it was frustrating to everyone that the Applicant could not be discharged home in a timely fashion. However, the home contained too many obstacles and barriers for the Applicant to live there independently. The home was acceptable for visits, but it could not be a long-term discharge option without significant renovations.

The physician referred to his letter dated March 5, 1991, marked Exhibit 2 to the hearing. The letter confirmed that the Applicant could not be discharged into his family home unless that home was significantly modified.

The physician testified that the Applicant had demonstrated extraordinary motivation. He returned to university. However, he required suitable accommodation in order to be discharged from hospital. The physician referred to his letter dated April 11, 1991,

marked Exhibit 3 to the hearing, which indicated that an accessible apartment was a suitable interim measure for the Applicant's discharge. The physician testified that remaining at the Parkwood Hospital inhibited the rehabilitation goal of fostering the Applicant's independence and this was why it was crucial to discharge the Applicant as soon as possible.

The physician referred to Exhibit 4, a letter dated April 11, 1991, explaining that in view of the Applicant's rehabilitation needs and goals, home renovations were required. In Exhibit 4, the physician considers the various alternative plans for the home renovations, and gives his opinion that Option B of the first Future Care report (an addition to the home) is the most preferable option to accommodate the Applicant and his family.

The physician testified that the Applicant was discharged towards the end of April 1991, when he was released to an apartment which he shared with his sister. Although he can be functionally independent in optimal circumstances, the apartment to which the Applicant was discharged has barriers which inhibit his ability to manipulate his surroundings. The Applicant can only live in the apartment with attendant care.

The physician referred to Exhibit 7, the first Future Care report dated March 1991. The physician identified Future Care Costs Associates as a registered company with accredited individuals serving on it, qualified to provide long and short-term assessments of disabled individuals. The organization looks at the necessary physical and rehabilitative modifications required to accommodate disabilities and impairments. The physician testified that he has often worked with this organization to assist in accommodating disabled individuals.

The physician testified that Future Care first became involved with the Applicant in order to facilitate his short-term discharge. The physician testified that the first Future Care report identified the barriers in the Applicant's home and indicated what would be required to overcome these barriers. The purpose of this report was to provide recommendations so that the Applicant could be discharged from hospital as soon as possible.

The report concluded that an addition to the Applicant's home was preferable to the modifications proposed in the Winmar plan. An addition would provide the Applicant with an independent entry and exit to the home. It would allow for maximum integrity to the rest of the home and provide the Applicant with shelter for transfer purposes, i.e. for getting out of the car and into a wheelchair.

The physician identified the second Future Care report, dated May 7, 1991, marked Exhibit 8 to the hearing. This second report is more elaborate than the first report. It contains more details and looks at a longer time frame for the Applicant's rehabilitation. The physician indicated that the report lays out a basic, reasonable, and necessary plan for the Applicant's optimal rehabilitation, including recommendations for transportation. His view was that the report was conservative in many of its recommendations.

The physician stressed that the Applicant must get from his residence to school on time, and for this he cannot rely on public transit. Taxis are extremely costly. Therefore the Applicant requires his own vehicle. The optimum vehicle is a wheelchair-accessible van. In deciding what vehicle is appropriate for the Applicant, the physician testified that access and safety are the primary concerns. The Applicant requires a vehicle that he can transfer into alone from his wheelchair. Once he is in the vehicle, he should be properly restrained in it.

The physician referred to Exhibit 5, a letter dated April 11, 1991, explaining the Applicant's need for a motor vehicle. He requires transportation for his education, rehabilitation and medical treatment sessions. The Applicant cannot use normal public transportation. It is not possible or practical for him to rely on the London handicapped public transit services which are limited, slow and unreliable. It is therefore the physician's opinion that the Applicant requires his own motor vehicle, properly adjusted for his handicap. The vehicle will promote the Applicant's rehabilitation since it will boost and promote his independence, and assist him to deal with the psychological effects of his injuries. It is the doctor's opinion that the Applicant requires a properly modified van and that such a motor vehicle is a necessary expense for his weekly treatment and ongoing rehabilitation.

The physician testified that although the Applicant is currently using a modified automobile on a regular basis, this is not ideal because of concerns about safety and access. The Applicant could fall in attempting a transfer and he could remain for hours without help. The Applicant currently must transfer from his wheelchair to the car using a hop-over manoeuvre which is analogous to the manoeuvre used on a pommel horse in a gym. A van is safer and more accessible for the Applicant. At present, the Applicant cannot use the passenger seat in the car which he drives because the wheelchair sits in the passenger seat.

The physician testified that in the circumstances, it is reasonable and necessary for the Applicant to have a vehicle. He needs a vehicle to achieve his vocational and educational goals. He also needs a vehicle for shopping, doing other errands, and for living as normalized a life as possible. The physician testified that in 1992 it is not acceptable for the Applicant to remain shut-in and have to wait to have his groceries delivered. He should be able to be integrated into the mainstream community as much as possible.

The physician testified that the Applicant has done extremely well, however, he continues to need to live with his family. He is currently living in sub-optimal conditions. Those conditions have fostered his dependencies on an attendant care-giver. The physician testified that the second Future Care report also addresses the Applicant's other requirements, including physical fitness equipment, and other rehabilitation needs. The physician referred to Exhibit 10, Dr. Geisler's report of May 8, 1991. This report supports all the recommendations made by Future Care. He also referred to Exhibit 11, the report of Dr. Clifford, and indicated that there were no inconsistencies between that report and the Future Care report. Dr. Clifford's report strongly recommends vocational rehabilitation counselling for the Applicant, and the Applicant can only attend vocational rehabilitation sessions if he has transportation.

Keith Charles Hayes gave sworn testimony on behalf of the Applicant. He testified that he is employed by the Parkwood Hospital and by the University of Western Ontario, and he is the Director of Spinal Cord Research programs at these institutions. In addition to

his full-time employment at the university, he has been asked to provide recommendations on future care costs and therefore together with his wife he established a small business called Future Care Costs Associates. In the past five years, this business has prepared about 200 assessments of future care costs and extraordinary living expenses for individual clients. The business identifies the future rehabilitation needs of disabled individuals and estimates the costs of rehabilitation. Such matters as home modifications, assistive devices, transportation costs, and the like are considered. The goal is to rehabilitate an individual so as to restore him or her as much as possible to the pre-injury status.

The witness explained that he has a Ph.D. degree. He does not have a medical degree. He explained that he is a medical scientist in the field of rehabilitation and that his research involves rehabilitation issues.

The witness bases his assessment of individual cases on medical documentation and input from physicians and others involved with the individual's rehabilitation and care. He always discusses the final report with the physician in charge, to make sure that the recommendations made are reasonable and necessary, and that nothing has been left out. The witness testified that his services were retained by the Applicant's solicitor. In this case, Dr. Allatt, the previous witness, was the attending physician.

The witness testified that he has worked with Dr. Allatt on numerous occasions. He testified that in this case both he and Dr. Allatt personally reviewed the first and second Future Care reports line by line to verify the appropriateness and accuracy of the recommendations. The witness was offered as an expert and was accepted as such.

The witness testified that the first Future Care report, Exhibit 7, was prepared in response to the Applicant's urgent accommodation needs. The second report is more comprehensive and itemizes all of the Applicant's future care needs.

With respect to the Applicant's transportation needs, the witness testified that the Applicant needs a vehicle in order to help him develop his independence. It takes the Applicant over two hours to get ready to leave his home. It takes him a long time to

dress and see to his personal hygiene. He should not be dependent on others for transportation.

The witness testified that the para-transit facilities in the City of London do not meet the Applicant's needs to get to and from the university promptly. The system is notoriously late. It must be booked 24 hours in advance and is not appropriate to accommodate spontaneous events. The witness testified that the system may be suitable for persons with occasional transportation needs, but this Applicant has regular transportation needs and he requires a vehicle to get out and about by himself.

The witness testified that Future Care costed a vehicle based on the recommendations of Dr. Allatt. Other options, even a motorcycle and a side car, have been discussed, but the other options are not appropriate. In the Applicant's case, the choice was between a modified car and a van. The witness testified that paraplegics can use either a modified car or a van. The considerations in making a choice are ease of access, safety, and climate conditions. The witness testified that he discussed the options with Dr. Allatt, who suggested that a van was more appropriate. One of the considerations for choosing a van was that the Applicant plans to have a family, and a car reduces space for accommodating a family. Therefore, Future Care approached Richardson Access & Mobility Inc. ("Richardson"), and that company provided a cost estimate for a custom-modified van. The witness testified that the costing of the van was based on the recommendation of the Applicant's medical doctor.

The witness testified that the Applicant is currently driving his father's car, which has been modified for his use. One of the problems with using a car is that front seat space is taken up by the wheelchair and therefore passenger space is limited. As well, the Applicant must perform a rather tricky transfer manoeuvre to get into the driver's seat. He is capable of getting into the passenger's seat the same way, but, the witness testified, access to a van is, in general, much easier. With a van, the Applicant would avoid getting wet in the rain while he folded his wheelchair and moved it into the vehicle. A van would be much more convenient for the Applicant than a car. In a car, which is less roomy, it is much more difficult for the Applicant to twist and turn. The Applicant's

problems using a car will be exacerbated as he ages. It will be harder for him to perform the entry manoeuvre as he loses physical strength. The witness testified that if the Applicant does choose a car rather than a van, he should select a larger car. Most paraplegics prefer a two-door car because the doors of such a vehicle are larger. The witness testified that the choice between a car and a van also depends to a certain extent on how a person wants to be perceived. Some individuals may prefer a sporty-looking vehicle, regardless of the difficulty that they experience driving such a vehicle. However, the practical considerations must be space, ease of access, and safety. When considering these factors, a van is the most appropriate vehicle for the Applicant. The witness testified that these days individuals using vans are not stigmatized. Many individuals drive cars as a matter of choice, but no specific stigma is attached to driving a van.

The witness testified that he is aware of one individual who drove a motorbike with a side car modification after he became paralysed. However, this is not appropriate for this Applicant. In any case, the Applicant has indicated that because of his experience he does not wish to drive a motorcycle again. The witness testified that a motorcycle would not be nearly as safe as a car or a van, and it would not be stable or useful in the winter months.

William George Richardson gave evidence on behalf of the Applicant. He testified that he owns a business called Richardson Access & Mobility Inc., which he has operated for five years. He is involved in the construction and renovation of homes to provide mobility and accessibility for disabled persons. He was formerly a tool and dye maker and did computerized design work. He then started manufacturing chair lifts and other items for disabled individuals.

The witness testified that he considered the Applicant's disabilities and decided what type of vehicle he required based on those disabilities. He determined that the Applicant required a vehicle with straightforward hand controls and a transfer system for getting from the wheelchair into his vehicle.

The witness felt that a full-size conversion van was the best vehicle for the Applicant. He referred to the document marked Appendix F in the second Future Care report, which provides his recommendations and estimates for a vehicle. He recommended a straightforward heavy-duty Ford cargo van, which could be custom-modified for the Applicant. The vehicle must be heavy-duty because of the lift to be installed. He recommended that power doors and locks, and a swing-away, front-loading wheelchair lift be installed in the vehicle. With such a lift, the Applicant could park his vehicle in a regular parking spot.

He recommended lowering the floor of the vehicle because the Applicant might be driving the car from the wheelchair. He also recommended raising the door so that the Applicant would not have to lean and duck to enter the vehicle. He recommended raising the roof for maximum vision and head room. He recommended seat swivels. The witness testified that the vehicle comes as a cargo van. The windows and all the optional features are added. The witness testified that to design and customize such a van is a costly and labour intensive process, and not many companies do it.

The witness explained that the type of vehicle required for a disabled individual depends on the activity of the person using it. The more the vehicle is used, the easier it must be to use. If it was raining or snowing, the Applicant could have difficulty with his transfers, and problems with his wheelchair sliding. Therefore he felt that a van was optimal for the Applicant. It would be an ideal unit because it could go anywhere, with a minimum of trouble. The witness testified that in his view a car was of limited use to the Applicant and is therefore less desirable.

The witness testified that mini vans were becoming more popular with many paralysed individuals. He has a conversion package for a mini van which is almost as convenient as a regular full-size van. The witness showed pictures of a modified mini van fitted with a lift.

The witness testified that the converted mini van featured a lowered floor and centre section, and an automatic ramp which comes down to assist entry into the vehicle. The ramp can also swing out of the way so that passengers can enter the vehicle. The

witness testified that the price of the modified mini van was approximately \$38,000.00, plus sales taxes. He indicated that the life-expectancy of the mini van is approximately the same as the full-size van. The witness testified that the price of a full-size van, including the conversions and modifications, was approximately \$50,000.00, plus sales taxes, as set out in Appendix F to the second Future Care report. He testified that \$2,000.00 could be saved by eliminating the driver's well, but a \$1,200.00 powerpack would need to be added. The witness testified that a full-size van gives the Applicant the best mobility, but he indicated that a mini van is probably easier to drive. He testified that it was also possible to modify a car to install a "chair-topper" roof rack carrier for a wheelchair.

The witness testified that he does consulting and recommends modifications for a vehicle. The actual work customizing the van is done by a company in Kitchener. The witness testified that his company contracts with only this company in Kitchener. He does not tender the work out to the cheapest bidder because he has been dealing with the Kitchener company for a number of years. The witness testified that his company earns a percentage from the price of the entire van, which is built into the sale price.

The witness testified that he prepared the proposal for a vehicle for the Applicant, specifying the material costs of the modifications. In this case, the proposal was done without a driver's assessment, based on the second Future Care report, which indicates that the Applicant is an able driver who requires hand controls.

The witness testified that his proposal represented the optimum package for the Applicant. It does not require a driver's well, but does require a power-based chair for the Applicant. He confirmed that certain items could be reduced or eliminated, such as the lighting, the window dressing, and other items of trim. Certain driving accessories, costing about \$800.00, which have been added for the convenience of the Applicant, could be eliminated. The witness indicated that venetian blinds had been costed into the price of the van. Blinds would assist the Applicant if he were not driving. A safety package has been added, as well as an auxiliary air conditioner, which services the

back part of the vehicle. Such items as deluxe spoke wheel covers, an aluminum ladder, and a roof rack have also been added.

Robert Plows, the father of the Applicant, gave viva voce evidence under oath. He testified that he has been a police officer in the City of London for 21 years. He lives at 668 Headley Drive with his family, in a home which they have occupied for eight or nine years. He testified that the Applicant was injured on October 27, 1990, that he was immediately taken to Tillsonburg Hospital and then to the University Hospital in London. He was transferred to St. Joseph's Hospital for surgery, and subsequently, to the Parkwood Hospital for rehabilitation.

The witness testified that since the accident the Applicant has not lived at home, except on a short-term basis. He can only be accommodated at home when there are sufficient able-bodied persons to assist him. The witness testified that the house is a four-level side split. There is a family room on the second floor, a kitchen on the first floor, and the bedrooms are on the fourth floor.

The witness testified that the bedrooms, including the Applicant's former bedroom, are not accessible to the Applicant. Two able-bodied individuals are required to transfer the Applicant from the living space to the bedroom area. The Applicant then must stay in the bedroom until he can be carried down again by two able-bodied persons.

The witness testified that two steps lead from the threshold of the house to the path. The Applicant needs the assistance of two-able bodied persons to help him negotiate the threshold to enter or leave the house. The witness testified that before the Applicant had his accident, no one in the family had experience with the needs of handicapped persons.

The witness testified that he personally filled out the application for benefits on behalf of the Applicant. He provided the information to the Insurer and advised as to the Applicant's condition. The witness testified that he received the documents from the Morden & Helwig insurance agent, Christine Terashita. The application for benefits was initially delayed because information about the Applicant's income was required.

When the application for benefits was made, the Applicant was at the University Hospital. Ms. Terashita visited the Applicant's family at home to initiate the claim. Later, Ms. Terashita visited the home with the occupational therapist and the physiotherapist, in order to assess the home for the Applicant's return. Many obvious barriers were identified.

The Applicant's father testified that eventually Ms. Terashita introduced him to John Williams of Winmar Industries Inc. Mr. Williams prepared a plan and estimate for modifying the home. The witness testified that this plan caused concern because it did not address all of the accessibility problems in the home. It did not provide for modifications in the kitchen to make it usable for the Applicant. Neither were modifications planned for the rear, to enhance the Applicant's access to the home.

The witness testified that the Winmar plan provides an area for the Applicant to sleep and attend to his personal needs, using the existing family room. The rest of the family, therefore, would no longer have the use of the family room, and the family's living space would accordingly be reduced by about one third. The witness testified that the house is on the small side. It is a 35 year old home and the rooms are small. The witness testified that after seeing the Winmar plan he consulted his lawyer because of his concerns. He did not speak directly with the insurance agent but conveyed all his concerns to the lawyer. After this, the witness testified, he became involved with Future Care Cost Associates. He was becoming more educated about the Applicant's needs and also about the family's needs, as a support group. As the family became more educated, it became obvious that turning over space from the family room was not desirable, and would increase the pressure on the family unit.

The witness testified that the pressure on him started from the moment of the accident. He was present when the Applicant had the accident. The Applicant was taken to hospital, and the trauma increased when the father realised the extent and lasting effect of the injuries on the Applicant, and on the rest of the family. The father testified that the family was fortunate in that the second daughter is a nurse who had been working in England. She returned home and has been a constant companion to the Applicant. This

released the father and his wife from the constant demand to attend to the Applicant's needs. The father testified that he took a leave from work to deal with the Applicant's requirements for the future.

The Applicant's father testified that he felt that the family was failing the Applicant because he could have been discharged at the end of January 1991 if the house had been modified. He had hoped that the home would be modified prior to the Applicant's release date.

The father testified that the Applicant's sister made extensive efforts to locate alternate, wheelchair-accessible accommodations for the Applicant. Many apartments were viewed, but relatively few were accessible. It was difficult to rent an accessible apartment because, in many cases, the owners wanted leases of a year to two years. This was not appropriate because the Applicant planned to ultimately return home. The father testified that the pressure was mounting as the Applicant remained in hospital and was not able to be discharged.

The father testified that the Applicant was eventually discharged from hospital on April 26, 1991, into an apartment which he shared with his sister. The Applicant was able to function in the apartment with the assistance of his sister. The Applicant's father testified that the Applicant and his sister rented the apartment jointly on a sub-let, and subsequently took it on a month-to-month tenancy. The apartment rents for \$948.00 a month, plus charges for utilities, hydro, water, telephone and cable T.V.

The witness explained that the Applicant was a student at the time of the accident. He had previously worked in the Air Force. He was released from the Air Force and planned to live at home to complete four years of education.

The father testified that throughout the time of his convalescence, no one apart from the Applicant's family members has cared for him. He testified that someone quite strong is needed to carry the Applicant up or downstairs because the Applicant and his wheelchair must be lifted. The father testified that the whole situation is stressful. He feels strongly that the family should have been able to get the Applicant home, where

they could have assisted him as required, as soon as possible. The father testified that he has provided the Applicant with a leased motor vehicle. Before that, other people were responsible for transporting the Applicant.

The father testified that the Applicant returned to university as a patient at the hospital. He attended school four days a week, every day except Thursday. He sometimes had to be transported back to the hospital between classes. Someone would have to go to the hospital, get the Applicant to school, wait, return him back to the hospital and then back to school, and back to hospital again. That occupied an entire day for three out of four days. On the other day, the Applicant had just one class, which required only one return trip. The Applicant also had to be transported for errands and shopping. At that time, the Applicant was transferring into a car by sliding on a board from his wheelchair to the car seat. The wheelchair then had to be disassembled and placed in the trunk of the car.

The Applicant's father stated that the Applicant has made remarkable progress and has conceded as little as possible to his injury. He has learned to dress himself, although putting on his pants and underwear is a task. The Applicant must prepare extensively to attend school. Now that he is proficient in attending to his needs, he requires an hour and a half to get ready to leave the apartment.

The father felt that the Insurer could have done more, sooner, to assist the family to deal with a very difficult situation. The first contact with the Insurer was through Christine Terashita. The father testified that he met with Christine Terashita and with Lynda Vokey, the therapist at Parkwood Hospital, before the end of 1990. Lynda Vokey and the physiotherapist came to the house to assess its accessibility for a short-term visit. After that, the family met with Lynda and Christine to discuss home modifications.

The Applicant's father testified that he believes the Insurer has the responsibility to meet with the family to discuss changes in the house. He testified that he organized the meeting at the home in January. He arranged to allow the Winmar construction people into the home. He had no direct contact with anyone other than Christine Terashita. Such contact, he stated, was at his initiation.

The father testified that he was first contacted by rehabilitation experts on behalf of the Insurer in September 1991. He referred to Exhibit 12, a report by Associative Rehabilitation Inc., which represented the first contact of the Insurer's rehabilitation expert. This report was prepared in the context of the arbitration hearing. The witness testified that he saw only the insurance agent and the Winmar representative before retaining his lawyer. He saw other sub-contractors, but never spoke with Dr. Geisler or Dr. Clifford, the Insurer's other rehabilitation experts.

In cross-examination, the father was questioned about Exhibit 9, the report of Lynda Vokey addressed to Morden & Helwig. He indicated that he had never seen it before. He confirmed that the Winmar plan was dated February 21, 1991. He stated that his solicitor did not become involved until after he received the Winmar estimate. He stated that he did not sign the Winmar document authorizing construction to begin because he felt it was not complete, and he had to consider it before signing it. He did not indicate his concerns on paper, but communicated them to his solicitor. He stated that towards the end of February he was beginning to have doubts about the Insurer's intentions. He did not wish to jeopardize his position by prematurely signing a document which appeared deficient. However, he indicated that an alternate plan of renovation for the house had not yet been developed. He had not retained any consultants, but had left it to the Insurer, since Ms. Terashita had already contacted Winmar.

The Applicant's father stated that by February he was becoming concerned because of the mounting delay. He was beginning to feel that perhaps he was not being dealt with fairly. At this point, therefore, Future Care Costs Associates was retained. He does not recall when he received the first Future Care report. The father confirmed that he never directly advised the Insurer that the Winmar plan was not acceptable. He did not get back to Ms. Terashita or advise her that he refused to authorize the Winmar modification plan. He gave his solicitor his copy of the estimate and advised the solicitor of his concerns.

The father testified that he received the first Future Care report through his solicitor. At that time, he knew that the matter was being referred to mediation, and therefore he did

not provide the Future Care report directly to the insurance agent. He left all communication to his solicitor. He testified that he had elected the "B" option recommended in the Future Care report, which provides for an expansion of the home rather than modification of the existing premises. He did not advise his lawyer to instruct the Insurer of his choice. He was not at first aware that two separate reports were being done by Future Care. However, he consented to two separate reports being prepared, on the advice of his solicitor.

The father testified that before the arbitration hearing, he was not aware that the Insurer was prepared to pay for the "A" option outlined in the first Future Care report. He did not know that the Insurer was willing to do more than what was outlined in the Winmar plan. He did not know whether his concerns had been directly communicated to the Insurer and had been rejected. He was not aware that the Insurer wished to consider Option "B". His primary concern was that the Winmar plan was not adequate and should not be implemented.

The father testified that he does not remember exactly when he received Exhibit 8, the second Future Care report. It was in early June, and he took it with him to read on his vacation. He only realized that the contractor had provided a lower estimate for the "B" option after the hearing had been commenced. He testified, that prior to the hearing, he had built a ramp at the rear of the house to facilitate access, at a cost of \$70.00. He did not submit this cost to the Insurer for reimbursement. He did not directly submit to the Insurer his claim under Option "B". The father testified that the rental bills for the Applicant's apartment were submitted to his solicitor, and he does not know when these bills were forwarded to the Insurer.

The father testified that he was hoping that all the issues could be dealt with at mediation. However the sixty-day mediation period went by without anything happening, so the family ended up pursuing arbitration. The father testified that he was not advised why nothing had happened in mediation.

Christine Terashita gave sworn testimony on behalf of the Applicant. She testified that she is an investigator with Morden & Helwig Limited, a firm of insurance adjusters. The

term "investigator" is used to describe an individual who is not fully licensed as an adjuster. She testified that she had been an independent adjuster with a company from Toronto for one and a half years. Her job as an investigator for Morden & Helwig is to investigate, negotiate and report to the Insurer. She makes recommendations with respect to settlement.

The investigator testified that she had been retained by the Insurer to deal with the Applicant's claim on November 5, 1990, when she received the facts regarding the claim and started an investigation. The head office of the Insurer is located in Montreal. The Insurer had identified the Applicant, his address, the vehicle, the coverage, the date of loss, and the description of the accident. The accident report indicated "insured lost control" but no description was given of the Applicant's injuries. Normally the investigator contacts the insured to find out what happened. She tried to contact the Applicant but could not reach him. She therefore wrote to the Applicant on November 5, 1990 at his home address, requesting that he contact her. At that time, she stated, she had no idea what had happened to the Applicant or where he was.

The investigator testified that she made a telephone call on November 9, 1990 and spoke to the Applicant's father. She learned that the Applicant was in hospital for surgery. She reviewed the accident claim forms with the Applicant's father.

She testified that she met with the Applicant's father on November 12, 1990, at the residence. She obtained an accident and injury report and a diagram of the accident. She reviewed the Applicant's injuries with his father, although not in detail. The same day, she reported to the Insurer stating that she had met with the Applicant's father, and indicating that a more detailed report would follow. She subsequently prepared a full, detailed report dated November 14, 1990, which indicated that she received the injury report advising that the Applicant had been paralysed from the waist down. At this point, a third party report was not yet available. A signed statement had to be obtained from the Applicant's father, because the Applicant was not capable of giving a signed statement, and his father was acting on his behalf. The Insurer ordered a police report to find out about the third party's situation since it was concerned about its liability.

The investigator testified that on November 29 she issued another report to the Insurer indicating that she was still awaiting a statement and advising that the Applicant had retained a solicitor.

The investigator testified that she met with the Applicant's father on December 1, 1990, to obtain a signed statement and to answer certain questions about medical benefits. They discussed air fare for the sister from out of town and other disability benefits, and also the confirmation of income data which was required. The Applicant's father was concerned about expenses which had been incurred. He submitted claims for ambulance fees, foot drops, and medical devices. The investigator promised to check about the air fare for the sister. She did so, and on December 1 advised the father that the Insurer would pay reasonable expenses for travel.

The investigator testified that she received Exhibit 1, the Application for Accident Benefits, on or about November 30th. This was submitted to the Insurer, together with a statement from the Applicant's father received on December 1st, and a medical authorization. The medical authorization allowed the adjusting firm to obtain medical information about the Applicant's injuries. This information was required so that the Insurer could follow and assess the Applicant's condition. The medical authorization was signed by the Applicant's father on December 1, 1990. At that time, the Insurer was awaiting the completed accident benefit forms. The investigator stated that she understood that the medical expenses would be substantial. She advised the Insurer that the Applicant was paralysed and therefore funds were requested. She submitted a further report to the Insurer on December 4, 1990, a photocopy of which was marked Exhibit 14 to the hearing.

The investigator testified that on December 4, 1990 she received Exhibit 1, the application for benefits, completed by the Applicant's father. This was faxed to the Insurer on December 5th, with a request for disability benefits at the rate of \$185.00 per week, pending confirmation of the Applicant's income. She then sent a letter to the Applicant enclosing an authorization for disclosure of employment information.

A further report was sent to the Insurer on December 12, 1990 again requesting disability benefits at the rate of \$185.00 per week, and also advising that the adjusters were awaiting reimbursement for medical benefits. The investigator testified that on December 17, 1990 she sent a further fax requesting that the Insurer advise of disability benefits, and indicating that the reconstruction of the Applicant's house was now being looked into.

The investigator testified that she communicated at all times with the Claims Supervisor for the Insurer, Jack Grammas. She advised Mr. Grammas that renovations of the Applicant's home were being investigated and that she would advise about amounts. This was as a result of her conversations with the Applicant's father, who requested that access be looked at since the Applicant would be returning home.

The investigator testified that she believed that home renovations were payable to a certain degree. She stated that it was her role to make recommendations to the Insurer, who would decide on those recommendations.

The investigator testified that she received another telephone call from the Applicant's father on December 17, 1990 advising that the Applicant was coming home and stating that reconstruction of the home was required. The investigator testified that she had been to the home and knew it was not ramped or accessible to handicapped individuals. On the same day, therefore, she communicated with Winmar Industries, a general contractor in the London area. She stated that each adjuster has certain contractors that they use on files, and Winmar was a contractor that she used. She advised John Williams of Winmar that work was required for a handicapped home. He indicated that the company could do the work. He said they would look at the home and see what was necessary for access. The investigator testified that she was waiting for confirmation from the hospital as to what was required to provide access to the home, by way of temporary renovations, to allow the Applicant to visit.

The investigator testified that she met with the Applicant's father on December 17, 1990 to update the ongoing disability benefits. On December 19, 1990, she spoke with Dave Weaver, a social worker at the Parkwood Hospital. He advised that rehabilitation

workers and occupational therapists had assessed the home, and that a report would be forthcoming in the new year, outlining the necessary renovations. It was decided that all the concerned parties should meet to discuss the renovations. At that point, the investigator testified, she had not received anything in writing about the access requirements. The Applicant's father was concerned that everyone meet to develop a plan of action.

The investigator testified that she sent another fax to the Insurer on December 19, 1990. This fax dealt with the disability payments and medical bills that were still outstanding. She received a cheque from the Insurer on December 20, 1990, providing monies for ten weeks' disability benefits, up to and including the week of January 1, 1991. On December 27, 1990, a cheque was received for the medical and ambulance expenses. On December 20, 1990, she also received a fax from Dave Weaver with the report from Lynda Vokey regarding accessibility. At that point, the issue was dealt with as a temporary access problem.

The investigator testified that she sent a seventh report to the Insurer on December 27, 1990, marked Exhibit 16 to the hearing. She advised the Insurer about the contact with Dave Weaver. She advised that they had requested an estimate from Winmar, since at that point the Applicant was leaving the hospital on day passes.

The investigator testified that a meeting was organized on January 8, 1991, at the home of the Applicant. The Applicant's father, John Williams from Winmar, Lynda Vokey, the occupational therapist from Parkwood Hospital, and she herself were present at the meeting. A home assessment had been done. Lynda Vokey advised that the front and back of the home had to be ramped, and doorways had to be widened for wheelchair access. There was a discussion about remodelling the downstairs of the home so that the Applicant could live there.

The investigator testified that at that time she did not know that the Applicant was coming home imminently. At the meeting, John Williams of Winmar went through questions and answers with the Applicant's father in order to figure out the basic accessibility requirements. Mr. Williams took measurements and made various

suggestions for renovations. Winmar undertook to produce a written plan and estimate for the renovations.

The investigator testified that further discussions took place about a chair lift and elevators on January 19, 1991. Elevators would have to be ordered separately and estimates for their cost and the cost of instalment would be required. At or around that time, the investigator learned that the Applicant was returning to school on a modified schedule.

The investigator testified that she spoke with the Applicant's father again on January 21, 1991, when they discussed a roll-in shower unit. This discussion arose because the Applicant's father contacted her office with concerns about the downstairs bathroom. On January 23, 1991, she was advised by the Applicant's father that he had different ideas about construction of staircases and he indicated that someone was going to his house to check that out.

The investigator sent an eighth report to the Insurer on January 25, 1991. She advised the Insurer that the Applicant was attending university on a limited basis. She had met with the occupational therapist and reviewed the necessity for modifications to the home. At that time, they were waiting for the Winmar plan and were engaged in discussions about elevators and stairwells. The general contractor had to contact suppliers for the elevator equipment. Mr. Williams was attempting to get the numbers together to come up with a reasonable estimate.

The investigator testified that she spoke to the Applicant's father again on February 5, 1991, when he advised her that the Applicant had gone to Kitchener for a driving assessment. It was known that the Applicant would not be able to drive a motorcycle and that he would need a specially equipped motor vehicle to get around. At that time, in early February, the investigator was aware that efforts were being made to provide a motor vehicle for the Applicant. She received the Winmar plan on February 5, 1991 and she faxed it to the Insurer on February 5th and 7th, together with a covering note marked "urgent" (Exhibit 17).

The investigator testified that she marked the fax "urgent" because she wanted the Insurer to look at it as soon as possible. She was aware that renovations were needed in a hurry, as the Applicant would soon be discharged from hospital.

She sent another fax dated February 8, 1991 to the Insurer (Exhibit 18) and also marked it "urgent". This was to obtain the authorization for the estimate. The investigator testified that on February 11, 1991, she tried to contact the Insurer several times by telephone. She made five telephone call attempts and left a message for Jack Grammas the sixth time. Mr. Grammas called back on February 13, 1991. He advised the investigator that he required a competitive estimate for the work. The investigator advised Mr. Grammas that she usually used Winmar and pointed out the urgency of the matter to him, but he stated that he required another estimate. Therefore, the adjuster contacted First General Contractors on February 13, 1991.

The investigator testified that nothing was wrong with the Winmar estimate, but the Insurer wanted a competitive estimate for pricing purposes. First General sent someone out as soon as possible. On February 18, 1991, the investigator sent another report to the Insurer (Exhibit 19) enclosing the estimate from Winmar and the competitive estimate from First General. The First General estimate was about \$1,500.00 less than the Winmar estimate. On February 20, 1991, Mr. Grammas authorized the renovations to proceed according to the Winmar estimate. At that point, the Insurer indicated that it was willing to pay for modifications to the home, but not for the purchase of a car. The investigator testified that she advised the Applicant's father of this. She also communicated with Winmar on the same day, and advised that someone should contact the Applicant's father, to start the repairs in accordance with the estimate.

The investigator testified that the Applicant's father phoned back about the van on February 21, 1991. He advised that the cost of the vehicle was between \$14,000.00 and \$22,000.00, and that it must have hand controls and air conditioning.

The investigator testified that she advised the Applicant's father that the Insurer was willing to pay for modifications to a vehicle but not for the actual purchase of the vehicle.

The investigator could not remember exactly when she advised the Applicant's father of this.

The investigator testified that on March 7, 1991 she again spoke with the Applicant's father. At that time, he stated that he was concerned with the Winmar estimate, and did not wish to authorize the work because certain items were lacking. He advised the investigator that he had turned the matter over to his lawyer. The investigator testified that on the same day she received correspondence from the Applicant's solicitor dated March 4, 1991, enclosing the first Future Care report. The correspondence indicated that the family was concerned about the Winmar proposal.

The investigator testified that on March 7, 1991 she sent another report to the Insurer, enclosing the correspondence from the Applicant's solicitor, and the Future Care report. Her March 7th report was marked Exhibit 21 to the hearing.

The investigator testified that on March 15, 1991 she received a note indicating that the parties were proceeding to mediation, regarding the issue of the home modifications. She stated that she was still involved with the Applicant's claims for the purpose of the accident and disability payments. She also arranged for transportation expenses to be paid. She then turned the file over to the Insurer's lawyer. She had no further involvement with the case and simply paid the bills.

In cross-examination, the witness testified that she never saw Exhibit 8, the second Future Care report. She also testified that she had never before handled a catastrophic claim under the No-Fault Benefits Schedule. She testified that she felt she handled the claim quite expediently, and even worked on the file one Saturday. She stated that she was assisted by the Insurer, and by the others involved, in a timely fashion. She felt that the Winmar contractors performed appropriately. She testified that she was in contact with the Applicant's family on a daily basis.

Submissions of the Applicant:

The Vehicle:

Counsel for the Applicant submitted that in a first-party insurance contract the Insurer and the insured must deal with each other in good faith. In a catastrophic case, there is a higher than usual responsibility that the Insurer respond to the needs of the Applicant in good faith. Counsel submitted that section 6 of the No-Fault Benefits Schedule states that the Insurer "will pay" all reasonable expenses. The wording is "pay" rather than "provide" and the only limit on reasonable expenses is the monetary limit set out in the legislation. Therefore, counsel submitted that the issue to be decided is what is reasonable in today's society, as a result of the accident.

Counsel indicated that the home renovations were provided for under subsection 6(e) of the No-Fault Benefits Schedule and verified that a consent order has now been signed regarding the home renovations.

Subsection 6(f) allows for other goods and services which the insured person requires, and does not use the words "necessary" or "essential". In contrast, counsel submitted that the prior legislation spoke to services and supplies that were, in the opinion of a physician, essential for treatment and rehabilitation.

Counsel therefore argued that the current No-Fault Benefits Schedule broadens the benefits that are allowed, beyond those allowed under the former legislation.

Counsel indicated that in this case there was no issue as to the Applicant's eligibility for no-fault benefits. The Insurer obtained a completed application for benefits. The company learned of the facts and background of the accident very quickly. Nothing was missing in the application for benefits, and therefore the Insurer should have provided benefits promptly.

Counsel submitted that the Insurer refused to provide a vehicle, but would only provide modifications to an existing vehicle, which the Applicant was obliged to acquire for himself. Counsel argued that there is clear evidence that the Applicant requires a

vehicle. No evidence has been led to the contrary. Dr. Allatt recommended a van, a vehicle which he felt would best allow the Applicant to function independently. He testified that a van was a basic requirement, and not a luxury. Dr. Clifford's report suggested a car might be suitable, but Dr. Clifford is not as highly qualified as the experts retained by the Applicant.

Counsel submitted that the Applicant was entitled to the device that would best guarantee his independence. The Applicant should not have to depend on others to transport him, and the para-transit system in the City of London is inadequate. The Applicant's physician has recommended what is optimal in the circumstances. Although the Applicant can "make do" with a lesser vehicle, the legislation does not require him to "make do".

Special Award:

Counsel submitted that the Applicant was discharged from hospital at the end of April 1991 rather than at the end of January 1991, and this delay cost the taxpayer almost \$400.00 a day in hospital charges. Counsel submitted there was a heavy onus on the Insurer to participate actively in the Applicant's rehabilitation, and to advise him of the insurance benefits available, because of the catastrophic nature of his injuries.

Counsel pointed out that the Applicant's recovery had surprised everyone. The temporary renovations first contemplated soon became out of date. Counsel submitted that the Insurer must respond more quickly and more diligently in catastrophic cases because in these cases it is crucial that the insured be provided with services as soon as possible. Counsel argued that the Insurer cannot treat this as an ordinary business case.

Counsel submitted that the Insurer was made aware of the urgency of the Applicant's needs by the investigator's reports, marked "urgent" and also by means of the Mediator's report. The Insurer knew of the pressing need for home renovations. This need was made clear in every document, including the second Future Care report (Exhibit 8), which the Insurer indicated it had only received in September.

Counsel also submitted that the Insurer's position that it does not have to provide a vehicle is unreasonable and has engendered an unreasonable delay. Counsel argued that the Insurer should have provided for all the requirements of the Applicant right away. The Applicant's father paid for the Future Care assessment, a service the Insurer should have provided.

Counsel submitted that if the Insurer had spent enough time on this matter, it would have been resolved much sooner. He submitted that the Insurer has acted indifferently if not inattentively. Such indifference warrants a message in the form of a special award of costs.

Submissions of the Insurer:

1. Motor Vehicle

Counsel submitted that the basic philosophy of the legislation is to reimburse and compensate individuals for expenses which they have incurred as the result of an accident. Accordingly, the Insurer's obligation to pay for an item depends on the liability incurred by the insured person.

Counsel submitted that Section 6(1)(d) of the No-Fault Benefits Schedule sets out the Insurer's liability for transporting a claimant and covers the Insurer's liability for all transportation costs. Therefore, Counsel argued, the Insurer cannot be made liable to purchase a car for an insured to serve for other transportation needs.

Counsel referred to subsection 6(1)(f) which deals with other goods and services required because of the accident, and argued that the word "required" must have a particular meaning. Counsel submitted that subsection 6(1)(f) must refer to goods and services required to make sure that the rehabilitation is successful, other than the goods and services listed in subparagraphs (a) to (e). Otherwise, subparagraphs (a) to (e) would be redundant.

Counsel submitted that any expense allowed under subsection 6(1)(f) must be both necessary and reasonable for the Applicant's treatment and rehabilitation. Counsel

argued that the expense must be such that without it, the rehabilitation program of the Applicant would fail. Counsel submitted that in this case, the Applicant's rehabilitation would not fail without a car, although he conceded that a car is desirable in terms of restoring the Applicant's dignity. However, counsel argued, alternative means of transportation are available.

Counsel referred to the case of Stewart v. Allstate Insurance Company, 510A.2d 131 (NJ 1986). He argued that the present case can be distinguished from that case because in the present case we have not had evidence that without a vehicle the Applicant's rehabilitation program will fail.

Counsel also raised the issue whether there is a liability to provide a van or something less. He submitted that the Applicant currently uses a car, and that the objectives of transportation are equally met by a car or a van. Counsel submitted out that a car can be converted for the use of a handicapped person for \$800.00. A car can be bought for \$20,000.00. The Insurer is currently prepared to pay the cost of renovating a car that the Applicant buys, although it does not feel it is obliged to do that.

Counsel submitted that if it were determined that the Insurer should purchase a car for the Applicant, it should also be declared that the vehicle is owned in the name of the Insurer, or in the Applicant's name in trust for the Insurer. He submitted that the no-fault legislation is compensatory in nature. It is meant to compensate an injured for expenses for which he is out of pocket. Counsel argued that to bestow a capital gain on the Applicant is to over-compensate him, and that this issue also stood in the way of an agreement with respect to the home renovations. Counsel submitted that if I, as arbitrator, order that a van or car must be purchased, I must also then deal with the capital gains issue. Counsel referred to the American cases of Reed v. Citizens Insurance Company of America (unreported decision of Hon. R.L. Tahoven, issued June 11, 1990, County of Clinton, Michigan), and Manley v. Detroit Automobile Inter-Insurance Exchange 388 N.W.2d 216 (Mich.1986), as shedding light on the issue of capital accretion.

2. Special Award

With respect to the issue of the special award under Section 282(10) of the Insurance Act, counsel noted that the Insurer retained an investigator on November 5, 1990, approximately one week after the accident, which had occurred on October 27, 1990. Counsel submitted that the investigator displayed initiative and diligence in assisting the Applicant and his family. Within three weeks of learning of the catastrophic nature of the injuries, she engaged a contractor, and met with the contractor and counsellors to define the Applicant's need for home renovations. Counsel pointed out that the medical report attached to the Application for Benefits dated November 30, 1990 (Exhibit 1) states in paragraph 6 that it was anticipated that the Applicant's in-patient treatment would last between 3 and 12 months. The Insurer learned about the temporary discharge plans for the Applicant some time before December 17th.

The investigator met with the Applicant's father on January 8, 1991. The report from Winmar was produced on February 5, 1991. Counsel submitted that by February 21st the Insurer was prepared to pay for renovations according to the plan that had been developed. The cost was \$40,500.00 for temporary renovations. Up to this point, no formal claim for renovations had been submitted by the Applicant, nor had a claim for any of the other matters dealt with at this hearing been submitted. Counsel argued that the discussions about the renovations and the van had taken place as a result of the investigator's initiatives.

Counsel submitted that it took four weeks to develop the Winmar plan and three weeks to receive an answer from the Insurer. This was not a serious delay, especially in light of the fact that the Applicant was in hospital at the time, and the Insurer believed that he needed to remain in hospital. Counsel conceded that there was a delay in hiring a rehabilitation consultant, but the Applicant had not initially claimed or demanded that a consultant be retained.

Counsel submitted that the legislation creates a claim mechanism but that the insured has the responsibility to organize his own affairs and to make a claim. The Insurer only

has a duty to pay in proper cases, and cannot decide on the route that an insured should take.

Counsel submitted that on February 21, the Applicant's father declined to go ahead with the Winmar renovations because by then he had learned that permanent, more extensive modifications were necessary. Accordingly, he considered that temporary repairs were not appropriate. However, in the circumstances, the Applicant cannot blame the Insurer or its agents for the delay.

Counsel submitted that from the end of January 1991, the Applicant's family knew that the Applicant could be discharged from hospital. However, the Insurer was not advised of the accelerated discharge at the appropriate time, nor was it advised that the concept for renovations had changed. Counsel submitted that the Applicant produced a claim for the renovations for the first time on March 4, 1991. The Future Care report was a major piece of work that came as a complete surprise to the Insurer.

That report introduced the concept of the addition to the home for the first time. It arrived on March 7, 1991 together with a letter from the Applicant's counsel.

Counsel pointed out that the Applicant filed for mediation eight days after introducing the first Future Care report. Counsel submitted that the insurance contract requires good faith on both sides, and submitted that filing for mediation in these circumstances was done in bad faith.

Counsel submitted that the Insurer had genuine and legitimate concerns about the recommendations set out in the Future Care report, and that the Insurer reasonably required some time to consider and respond to those recommendations.

Counsel also submitted that delivery to the Insurer of the second Future Care report, with the revised estimate for the renovations, would have expedited the resolution of this matter. The parties agreed that the Insurer did not receive the second report until just prior to the arbitration hearing. Therefore the delay in dealing with this matter was partly attributable to the Applicant.

Counsel submitted that there is no evidence that the Insurer unreasonably withheld payment for the home renovations. The Applicant changed his mind about what he required done, and did not give the Insurer adequate time to respond.

With respect to the question of the van, counsel submitted that the Insurer refused to pay for the van as a matter of interpretation of the legislation, which is not unreasonable.

Findings:

1. Motor Vehicle:

The Applicant has claimed that he requires a suitably modified motor vehicle as a result of the accident.

In this case, the applicable section of the No-Fault Benefits Schedule provides as follows:

Section 6.

- (1) The insurer will pay with respect to each insured person who sustains physical, psychological or mental injury as a result of an accident all reasonable expenses resulting from the accident within the benefit period set out in subsection (3) for,
 - (a) medical, psychological, surgical, dental, hospital, chiropractic, nursing and ambulance services and the services of physiotherapists;
 - (b) prostheses, dentures, prescription eyewear, hearing aids and other medical or dental devices;
 - (c) rehabilitation, life-skills training and occupational counselling and training;
 - (d) transportation for the person to and from treatment, counselling and training sessions, including transportation for an assistant;
 - (e) home renovations to accommodate the needs of the insured person;
 - (f) other goods and services, whether medical or non-medical in nature, which the insured person requires because of the accident;

- (4) Subject to subsections (5) and (6), the insurer, before making a payment for an expense under subsection (1), may require the insured person to submit a statement signed by the insured person's qualified medical practitioner or psychological advisor stating that the expense is necessary for the insured person's treatment or rehabilitation.

The legislation stipulates, at subsection (1), that the Insurer will pay the reasonable expenses resulting from the accident, and proceeds to list the items and articles to be paid for. Subparagraph (f) refers to goods and services, whether medical or non-medical in nature, which the insured person requires because of the accident.

Subsection (4) provides that the Insurer may require a signed statement from a qualified medical practitioner stating that an expense is necessary, before making a payment.

Therefore, the criteria which must be met before I can find that the Insurer is liable to pay for an item (in this case a motor vehicle) under subparagraph 6(1)(f) are the following:

- (1) it must be a reasonable expense resulting from the accident
- (2) it must be required because of the accident
- (3) a medical practitioner must provide a signed statement that the expense is necessary for the insured's treatment or rehabilitation, if the Insurer so requires

I propose to deal with these criteria in reverse order.

The Applicant's physician, in Exhibit 5, provided a signed statement that a motor vehicle was necessary for the Applicant's treatment and rehabilitation in this case, thus satisfying the third criterion.

Therefore, I must determine whether a motor vehicle is required because of the accident, and whether it is a reasonable expense resulting from the accident.

It is clear from the evidence that, as a result of the accident, the Applicant now has extraordinary requirements for transportation. Prior to the accident, the Applicant's mobility was unimpeded. To meet his normal transportation requirements, the Applicant

could choose to walk, ride his motorcycle, use the public transit system, or the motor vehicles of others.

Because of the accident, the Applicant no longer has these options. He can no longer walk or ride his motorcycle to do normal errands. He cannot drive an ordinary car. The public transit system is inaccessible. The evidence at the hearing has satisfied me that the para-transit system in London is inadequate to meet the Applicant's daily, normal transportation needs. Unless the Applicant is provided with a suitably modified motor vehicle, he will have no independent means of transporting himself, and must remain dependent on others to meet his requirements for transportation.

Moreover, as a result of the accident, the Applicant's need for transportation has grown. The Applicant now requires transportation for his ongoing medical and rehabilitation treatments, as well as for his normal activities and errands.

Furthermore, the evidence of Dr. Allatt, which I accept, is that the Applicant's need for a motor vehicle has a psychological as well as a purely practical dimension. In effect, his reliance on others to transport him fosters his sense of dependency. Conversely, having his own motor vehicle would promote the Applicant's independence and autonomy, and thereby further the ultimate rehabilitation goal that the Applicant become as functionally self-sufficient as possible.

I do not accept the Insurer's argument that subsection 6(1)(d) limits its liability for transportation expenses. That section spells out the Insurer's ordinary liability for transportation costs associated with attending medical treatment, counselling or training sessions, which are required because of the accident. However, the Insurer's liability for transportation expenses cannot be limited under section 6(1)(d) in a situation where, as the result of an accident, an individual has extraordinary, ongoing requirements for transportation. Such a situation instead clearly falls within the ambit of section 6(1)(f), which deals with the requirement for goods and services in addition to those specifically enumerated in the subsections above.

Nor do I accept the Insurer's argument that it is only liable for expenses that have already been incurred by the Insured. The wording of section 6(1) is very clear: it states that the Insurer "will pay" all reasonable expenses, but does not require that the expenses be incurred. Had such a requirement been intended, I have no doubt that it would have been clearly stated in the legislation, as it has been, for example in the wording of section 6(2), which only obliges the Insurer to pay for expenses which have been actually incurred. That section states:

- (2) The insurer will pay with respect to each insured person who sustains physical, psychological or mental injury as a result of an accident an allowance that is reasonable having regard to all of the circumstances for expenses actually incurred by a spouse, child, grandchild, parent, grandparent, brother or sister of the insured person in visiting the insured person during his or her treatment or recovery.

(emphasis added)

The Insurer also raised the case of Stewart v. Allstate Insurance Co. (supra) in support of the proposition that a vehicle should only be provided to an individual where it is proved to be a necessary medical expense, from the point of view of the individual's rehabilitation.

In the Stewart case, it was found that the costs of a modified van equipped with a lift were indeed valid "medical expenses" under the New Jersey legislation, where the independent transportation was indispensable to the paraplegic's physical and emotional well-being. I note that this case was decided in favour of the claimant under American laws that are much more stringently worded than the Ontario law. The New Jersey legislation requires that the item in question be characterised as a medical expense before it can be claimed. This is in contrast to section 6(1)(f) of our legislation, which requires the Insurer to pay for goods and services "whether medical or non-medical in nature", so long as the need for them arises as a result of the accident.

Under the circumstances, I find that to meet his ongoing transportation needs and his other rehabilitation needs which have arisen because of the accident, the Applicant requires a suitably modified motor vehicle.

The question remains whether such a motor vehicle can meet the first criterion enumerated namely, that it meets the test as a reasonable expense resulting from the accident.

The word "reasonable", which is used in the legislation, has two different, although not unrelated meanings in this context. The Oxford English Dictionary defines "reasonable" as follows:

- (1) in accordance with reason; not absurd
- (2) within the limits of reason; not greatly less or more than might be expected; inexpensive; not extortionate; tolerable; fair

I find that it is in accordance with reason to provide the Applicant with a suitably modified motor vehicle. However, the expense represented by such a vehicle must also be within the limits of reason, in the sense that it should not be excessive or exorbitant.

I have concluded that the Richardson proposal for a custom-modified van is not "reasonable" in this secondary sense. The evidence of the expert witnesses, Dr. Allatt and Dr. Hayes, indicates that the Applicant reasonably requires a vehicle that has been modified for his use with hand controls. Apart from that, the evidence indicates that the vehicle should be fitted with a suitable wheelchair lift system, to allow for safety and ease of access to and from the vehicle, especially in inclement weather. I am also persuaded that the Applicant needs a telephone in the vehicle, for emergency situations.

It is clear from the evidence that the Applicant cannot and should not drive a modified motorcycle. The evidence also suggests that a modified car does not fully and adequately meet the Applicant's transportation needs, since he may experience problems in transferring from the wheelchair to the car, especially in bad weather conditions.

However, I am not persuaded that many of the features specified in the Richardson proposal are necessary to meet the Applicant's reasonable transportation requirements. The cost of optional items such as venetian blinds, deluxe spoke wheel covers, oak

trim, auxiliary air conditioner, and aluminum ladder and roof rack cannot be described as reasonable expenses resulting from the accident, for which the Insurer is obliged to pay.

Richardson's estimate for a full-size, custom-modified van was approximately \$50,000.00. He testified that a modified mini van would cost approximately \$38,000.00, and that functionally, it was almost as convenient as the larger vehicle. The mini van is fitted with an automatic ramp to assist entry into the vehicle. It is also large enough to accommodate the Applicant, his wheelchair, and other passengers if required. Richardson testified that the mini van is probably easier to drive than the full size van.

I therefore find that a mini van represents a reasonable expense resulting from the accident, and the Insurer is accordingly obliged to pay for that expense.

The Insurer has argued that if it is obliged to pay for a vehicle, the Applicant will somehow be over-compensated for his injuries: he will in fact gain, to the extent that he did not previously own a vehicle. The Insurer therefore requested that if I ordered it to purchase a vehicle for the Applicant, it should be allowed to retain ownership of the vehicle "in trust".

I have determined that a vehicle is a reasonable expense resulting from the accident, pursuant to subsection 6(1)(f) of the No-Fault Benefits Schedule. Section 6(8) obliges the Insurer to pay all such expenses under Section 6, to a maximum amount of \$500,000.00 with respect to each insured person. I see no reason to treat a motor vehicle differently from any other item - such as a wheel chair or prosthetic device -- which the Applicant reasonably requires as a result of the accident. The American cases referred to by counsel for the Insurer - Reed v. Citizens Insurance Company of America (*supra*) and Manley v. Detroit Automobile Inter-Insurance Exchange (*supra*) are not applicable to the present case, since they deal with entirely different insurance benefits conferred under a different statutory scheme. Therefore, the Applicant should be the legal owner of the vehicle.

2. Special Award

The Applicant has claimed a special lump sum award under section 282(10) of the Insurance Act, on the basis that the Insurer has unreasonably delayed or withheld payments. That section states:

If the arbitrator finds that an insurer has unreasonably withheld or delayed payments, the arbitrator, in addition to awarding the benefits and interest to which an insured person is entitled under the **No-Fault Benefits Schedule**, shall award a lump sum of up to 50 per cent of the amount to which the person was entitled at the time of the award together with interest on all amounts then owing to the insured (including unpaid interest) at the rate of 2 per cent per month, compounded monthly, from the time the benefits first became payable under the **Schedule**.

The Applicant claimed that the Insurer has a heavy onus to deal with catastrophic cases diligently and expeditiously. In this case, the Applicant claims that the Insurer has unreasonably delayed in commencing the necessary home renovations. It is claimed that the failure to deal with the home renovations on an urgent basis has detrimentally affected the Applicant's rehabilitation. His discharge from hospital was delayed and he has been unable to return to the family home pending the renovations. The Applicant also argued that the failure to promptly provide a motor vehicle constitutes an unreasonable withholding of payments contrary to section 282(10).

I find that the Insurer has not acted unreasonably in withholding payments in respect to the motor vehicle, since I am satisfied that the Insurer had a bona fide disagreement with the Applicant about its obligations under the legislation in this case. We are dealing here with relatively new legislation, and parties may fairly differ as to its interpretation. The Insurer's liability to provide a vehicle is not sufficiently plain and self-evident in this case, as to render its failure to do so the unreasonable withholding of a benefit, within the meaning of the section.

With respect to the home renovations, I also find that there has been no unreasonable withholding or delay on the part of the Insurer. The Insurer acted promptly in retaining

an investigator when it was notified of the accident. The uncontradicted evidence of the investigator shows that she dealt with the case diligently and conscientiously.

The question of home renovations appears to have been first raised with the Insurer by the Applicant's father, on or about December 17, 1990. At that time, temporary renovations were discussed, in anticipation of the Applicant's coming home for short visits. The investigator testified that she contacted her contractor about the home renovations on December 17, the same day that the issue was raised by the Applicant's father.

A meeting about the home renovations was organized by the Applicant's father and held on January 8, 1991. At that meeting, the contractor discussed with the Applicant's father and with the occupational therapist from the hospital the necessary modifications. As a result of that meeting, the contractor was authorized immediately to develop a plan and estimate of costs for the renovations. The plan was received by the investigator on February 5, 1991, and she immediately communicated with the Insurer for instructions on how to proceed. I am satisfied that up to this point, there was no unreasonable delay on the part of the Insurer.

The Insurer did not respond to the investigator until February 13, about one week later. At that point, a competitive estimate was requested. The competitive estimate was obtained on February 18, 1991, and on February 20, 1991 the Insurer authorized the renovations pursuant to the Winmar plan. I find that the delay of approximately two weeks, from the time the Winmar plan was received until the time that the work was authorized to begin, does not constitute unreasonable delay, in the circumstances.

The renovations did not proceed according to the Winmar plan because by the time the Insurer authorized the work, the Applicant and his family had changed their minds about what they felt was required. At that point, they had retained the Future Care company and were considering permanent rather than temporary alterations to the home. However, the Insurer was not advised of this change of plans until March 7, 1991.

The Applicant and his family did not choose to communicate directly with the Insurer about their new concept for the renovations. Instead, they chose to communicate through their solicitor. The first Future Care report was delivered to the Insurer under cover of a solicitor's letter. Approximately one week later, the mediation process was invoked. At this point, the Insurer indicates that it had not had the opportunity to fully consider and evaluate the Future Care recommendations, which contained proposals for home renovations then estimated at approximately \$90,000. I find that it was unreasonable for the Applicant to expect the Insurer to respond to the Future Care report within the space of about one week. I find that the Insurer reasonably required more time to consider the Future Care proposal, and that its failure to respond to that proposal when mediation was invoked does not constitute unreasonable withholding or delay.

I have no knowledge of what occurred during mediation, since that process is confidential. I do not regard the narrative and comments in the Mediator's report as either reliable or admissible evidence about that process, since it constitutes hearsay testimony about a confidential transaction. Therefore, I am disregarding the text of the Mediator's report for the purpose of these reasons.

I am aware that both sides were represented by legal counsel at mediation and that they failed to reach agreement with respect to the issue of the home renovations. Under the circumstances, I cannot find that the failure to reach agreement at mediation per se constitutes evidence of unreasonable withholding or delay on the part of the Insurer, sufficient to attract a penalty under the legislation.

I would observe that in this case it appears that the lines of communication between the parties broke down at a relatively early stage in the process. Their relationship became adversarial rather than co-operative, thus impeding effective dispute resolution. It was conceded that the Insurer did not receive the second Future Care report, containing the revised estimate for the renovations, until just prior to the arbitration hearing. It appears that the Applicant failed to deliver his rent receipts until after the second mediation was started. I appreciate that the Applicant and his family were operating from a position of

shock and distress. However, both sides have a duty to communicate with each other openly and in good faith. In this case, I have concluded that the failure to resolve the outstanding disputes cannot be attributed solely to unreasonable delaying tactics on the part of the Insurer. Therefore, I am declining to make a special award of costs.

The Applicant is entitled to an award for his ordinary expenses incurred in respect of the arbitration proceeding, as prescribed in Ontario Regulation 275/90 and Schedule 1 of the Dispute Resolution Practice Code.

Order:

1. The Insurer shall provide to the Applicant a suitably modified mini van, in accordance with the reasons herein.
2. A special award under Section 282(10) of the Ontario Insurance Act is not payable to the Applicant by the Insurer.
3. The Applicant is entitled to his expenses incurred in the arbitration proceeding under Ontario Regulation 275/90, and Schedule of the Dispute Resolution Practice Code.

Frederika Rotter
Senior Arbitrator

January 16, 1992
Date